

The booking/first antenatal visit

Assess the pregnant patient at the booking visit, ideally before 14 weeks. If already booked →161. If not done, check patient wants pregnancy and is eligible for Basic Antenatal Care → 158.

Assess	Note
Symptoms	Manage symptoms as per symptom page. Check if patient needs urgent attention → 159.
Estimated Delivery Date (EDD)	- Use first day of the patient's last period and SFH¹ to determine EDD and current gestation. - If unsure of dates and SFH¹ < 24cm, refer for ultrasound to confirm EDD.
Fetal movements	If > 20 weeks, ask about fetal movements: if reduced → 161.
TB	- If cough, weight loss, night sweats or fever, check for TB → 92. If patient has TB, refer to next level of antenatal care clinic. - If HIV positive, send 1 sputum sample for TB NAAT, even if no TB symptoms.
Mental health	- In the past 2 weeks, has patient: 1) been unable to stop worrying or thinking too much 2) felt down, depressed, hopeless? If yes to any → 143. - In the past 2 weeks, has patient had thoughts or plans to harm herself? If yes → 83.
Alcohol/drug use	Any alcohol/drug use is risky for baby. In the past year, has patient: 1) drunk ≥ 4 drinks ² /session, 2) used illegal drugs or 3) misused prescription or over-the-counter medications? If yes to any, discuss/refer.
MUAC ³ and BMI ⁴	- If MUAC < 23cm or BMI < 18.5 (or BMI < 23 if HIV positive): exclude TB → 92 and HIV → 110 and refer for nutritional support or to dietician, if available. Arrange advanced midwife/doctor review. - If MUAC ≥ 33cm or BMI ≥ 32, check diabetes risk below.
Abdomen	- Use tape measure to measure size of uterus from symphysis pubis to top point of uterus. This is the symphysis-fundal height (SFH). - Plot SFH according to gestation on SFH growth chart. Assess growth by looking where measurement falls in relation to percentile lines: - If SFH < 24 cm at booking, refer for ultrasound (ideally at 18-20 weeks), if facilities available. - If < 28 weeks and measurement above 90th percentile or multiple pregnancy likely, refer. - If SF below 10th percentile, check SF at next visit: if still below 10th percentile, refer/discuss for likely poor fetal growth. - If ≥ 34 weeks: palpate presenting part: if breech or transverse lie suspected, reassess at ≥ 38 weeks. If breech or transverse lie still suspected, refer. - If mass other than uterus in abdomen or pelvis, refer for assessment.
Vaginal discharge	If abnormal discharge, treat → 49. If watery discharge and no contractions, suspect prelabour rupture of membranes → 159.
BP	- If BP ≥ 160/110, manage and refer urgently → 159. - If ≥ 140/90, lie patient on left side for at least 1 hour, then repeat BP: - If repeat BP ≥ 150/100, refer same day → 159. - If repeat BP < 150/100, check urine dipstick for protein: - If ≥ 1+ proteinuria, refer same day. - If no proteinuria but headache, blurred vision or severe abdominal pain, treat for severe pre-eclampsia → 159. - If no proteinuria, educate about warning signs (persistent headache, blurred vision or abdominal pain), advise to rest/reduce workload and review in 1 week: - If BP at review ≥ 140/90, arrange same day doctor/advanced midwife review: treat for gestational hypertension → 163 and review weekly. If proteinuria/symptoms develop, refer urgently → 159. - Refer all at 38 weeks for hospital delivery.
Urine dipstick: test clean, midstream urine	- If leucocytes and nitrites in urine treat for likely complicated urinary tract infection → 59. - If proteinuria: - If trace or 1+ proteinuria with normal BP, reassess at next antenatal visit. If BP raised, manage above. - If ≥ 2+ proteinuria, repeat dipstick on a new urine specimen: if still ≥ 2+ proteinuria, discuss/refer. - If glucose in urine, check diabetes risk.
Diabetes risk	- Screen for diabetes only if risk factor⁵. - To screen for diabetes: give unfasted patient oral glucose 75g in 250mL water upon arrival at clinic. Check glucose after 1 hour: if ≥ 7.8, arrange further fasting test at next level of care clinic.

TB tests changing from 'Xpert Ultra' to 'TB NAAT'
(NAAT = nucleic acid amplification test and includes Xpert as well as newer TB tests).

Continue to assess the pregnant patient → 161.

¹Symphysis-fundal height. ²One drink is 1 tot of spirits, or 1 small glass (125mL) of wine or 1 can/bottle (330mL) of beer. ³Mid Upper Arm Circumference. ⁴Body Mass Index (BMI) = weight (kg) ÷ height (m) ÷ height (m). ⁵Glucose in urine, BMI ≥ 32, age ≥ 40 years, previous diabetes in pregnancy, family history of diabetes, previous unexplained stillbirth, previous baby ≥ 4000g, polyhydramnios, SFH large for gestational age, Indian ethnicity.